Recommended Diet and Avoided Foods for Gastrointestinal Problems

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Background

There is no single “stomach-problem diet” that works for everyone. Broad elimination plans are common, but the best diet depends on the cause of symptoms and should be tailored when possible. (1 source)

People with stomach or gut symptoms are often given broad, one-size-fits-all diet plans, such as exclusion diets, low-FODMAP diets, gluten-free diets, lactose-free diets, NICE-style advice, or wheat-free diets, but the evidence does not support the idea that one approach is right for all patients. Instead, diet should be guided by the likely cause of symptoms: if *H. pylori* is present, eradication treatment is needed, and if food intolerance or malabsorption is found, a personalized diet plan with a registered dietitian is recommended. This means the best advice is usually to match the diet to the diagnosis rather than assume every patient should avoid the same foods. (Schnedl et al., 2023)

Evidence

(Schnedl et al., 2023)

A personalized management approach in disorders of the irritable bowel syndrome spectrum.

> *"Up to date, dietary treatments of the IBS have neither been personalized nor diagnosed with sufficient scientific evidence. They have mostly been treated using 'one-size-fits-all' approaches. Such include exclusion diets, a low fermentable oligosaccharides, disaccharides, monosaccharides and polyols diet, and gluten-free diets, lactose-free diets, a diet recommended by the UK National Institute for Health and Care Excellence, and a wheat-free diet"*
>
> *".Additionally, if H. pylori is found, eradication therapy is mandatory, and if food intolerance/malabsorption is detected, an individual and personalized dietary intervention by a registered dietician is recommended."*

Core eating pattern and meal habits

A good starting pattern for many stomach or gut problems is to eat small, regular meals, eat slowly, and avoid very large or rushed meals. Also, drink enough fluid, mainly water or herbal tea, and keep a simple food diary to spot personal triggers. (13 sources)

- **Use a regular meal pattern.** Try to eat at set times instead of skipping meals or eating erratically. Several sources suggest **3 main meals**, with **2–3 snacks** or small meals through the day, depending on what feels best. (Spiller, 2021) (Ringstrom et al., 2009) (Radziszewska et al., 2023) (Cozma-Petrut et al., 2017)

- **Choose smaller meals more often.** Small, frequent meals can reduce stomach stretching and may be easier to digest than a few large meals. Avoid both **overeating** and, when possible, long gaps that lead to very large meals later. (Andreska et al., 2020) (Feng et al., 2021) (Sozen et al., 2023) (Malan et al., 2017) (Rosa et al., 2018) (Kuzmin et al., 2025)
- **Eat slowly and in a calm setting.** Take time to chew well and avoid hurried eating. Guidance for IBS and other stomach complaints repeatedly stresses that **how you eat** can matter as much as the food itself. (Spiller, 2021) (Dimitrova-Yurukova et al., 2022) (Pasqui et al., 2016) (Kuzmin et al., 2025)
- **Drink enough fluid every day.** A common target is about **1.5–2 liters a day** or about **8 cups**, mainly **water** and often **herbal tea**. This is especially important if diarrhea is present or if fiber is being increased for constipation. (Andreska et al., 2020) (Dimitrova-Yurukova et al., 2022) (Radziszewska et al., 2023) (Pasqui et al., 2016)
- **Prefer easy-to-digest foods when symptoms are active.** During flares or sudden symptoms, many papers suggest **light, soft, digestible foods**, and avoiding foods that are very rough, very hot, very cold, or strongly irritating. (Feng et al., 2021) (Sozen et al., 2023) (Zhang et al., 2024)
- **Keep a food and symptom diary.** This can help you find your own trigger foods instead of cutting out many foods at once. Self-monitoring is commonly advised before trying stricter changes. (Andreska et al., 2020) (Rosa et al., 2018)
- **Do not jump straight to broad food avoidance unless needed.** General healthy-eating advice and regular eating habits are often used first, while strict exclusion diets are usually reserved for selected cases and should be supervised. (Ringstrom et al., 2009) (Cozma-Petrut et al., 2017)

Evidence

(Spiller, 2021)

Impact of Diet on Symptoms of the Irritable Bowel Syndrome

> *"While this diet does recommend avoiding certain common foods often associated with symptoms such as spicy and fatty foods, alcohol, coffee, onions, cabbage and beans, it also focuses on eating style. Thus, it encourages regular meals, three times daily, along with 3 snacks and an emphasis on taking time to eat and doing so in a relaxed fashion rather than having hurried irregular meals that patients often report (Miwa, 2012). It also specifically advises against carbonated drinks and foods containing polyol sweeteners such as mannitol, as found in chewing gum"*

> *"dietary management should most economically start with the NICE guidelines."*

(Ringstrom et al., 2009)

Development of an educational intervention for patients with Irritable Bowel Syndrome (IBS) – a pilot study

> *"General advice was provided and the patients were encouraged not to avoid food. It was stressed that it might be useful to reduce the intake of some food items, but exclusion diets should only be tried in rare cases and under strict supervision. The focus was not on what the patients eat, but rather on how and when they eat. Regular eating habits are important, and three main meals and two to three snacks were recommended"*

> *".The importance of gas producing food items, including fibre intake, the importance of lactose and fructose intolerance and cooking methods, were also considered to be important issues to discuss in the group (Burden, 2001)(Dapoigny et al., 2003)."*

(Radziszewska et al., 2023)

Nutrition, Physical Activity and Supplementation in Irritable Bowel Syndrome

> *"According to the National Institute for Health and Care Excellence (NICE), basic dietary recommendations for patients with IBS should include regular meal consumption, avoiding meal skipping and large meals, drinking about 2 L (8 cups) of fluids per day (mainly water and herbal teas), limiting consumption of carbonated and alcoholic beverages, as well as reducing intake of caffeine, fats, insoluble dietary fiber (found in whole grains, cereal flakes, bran), resistant starch (present in processed and reheated foods), and gas-producing foods (including certain fruits) [19,(Bonetto et al., 2021). If these recommendations do not produce therapeutic effects, the next step is to implement a low FODMAP diet (Fermentable Oligosaccharides, Disaccharides, Monosaccharides, And*

Polyols), which involves eliminating foods rich in fermentable oligosaccharides, disaccharides, monosaccharides, and polyols [18]."

(Cozma-Petrut et al., 2017)

Diet in irritable bowel syndrome: What to recommend, not what to forbid to patients!

"general advice on healthy eating and lifestyle is recommended as the first-line approach in the dietary management of IBS. Standard recommendations include adhering to a regular meal pattern, reducing intake of insoluble fibers, alcohol, caffeine, spicy foods, and fat, as well as performing regular physical activity and ensuring a good hydration."

(Andreska et al., 2020)

Dietetic approach in patients with irritable bowel syndrome – a case report

"General recommendations include taking small meals throughout the day as well as probiotics to regulate the intestinal flora and reduce bloating and gas. To detect foods that exacerbate symptoms it is recommended that the patient keep a diet diary. In the phase of predominant diarrhea, one recommends adequate fluid intake, primarily water and herbal tea, restriction of caffeine-containing beverages, restriction of foods containing insoluble fiber (whole grain bread, nuts, seeds), restriction of eating fresh and dried fruit to 3 servings per day, reduction of starch intake and sugar free foods (containing sorbitol and xylitol), consumption of probioticcontaining products (yogurt, fermented dairy products) and avoiding fat-containing foods (chips, burgers, fried foods and sweets) as they can worsen diarrhea. In the predominant constipation phase, dietary fiber intake is recommended"

"Symptoms of constipation can be improved by combination of whole grains with fruits and vegetables. Oatmeal and linseed are good sources of soluble fiber, which help to soften the stool and ease the passage and can also help alleviate the symptoms of gas and bloating. Adequate fluid intake of at least 8 glasses of caffeine-free fluids per day (best water) has to be ensured."

(Feng et al., 2021)

Nursing Value Analysis and Risk Evaluation of Acute Gastroenterology Diseases

"Guide patients to pay attention to dietary hygiene, develop regular eating habits, eat nutritious and digestible soft food and a small number of meals, and avoid overeating, rough, overheating, overcold, and irritating food, etc. Eat a high-calorie, high-protein, vitamin-rich, low-fat, and digestible diet after a stable condition. Introduce foods beneficial to canker, such as peanut oil, bean products, cabbage, spinach, carrots, and honey."

(Sozen et al., 2023)

Nutrition and Helicobacter pylori infection in gastric disease

"In general, nutritional recommendations for patients with gastritis can be summarized as follows:"

"Fresh vegetables and fruits should be consumed every day to support vitamin C intake Meals should be eaten little, often and regularly. The consumption of dark tea, alcohol, coffee, roasts, spices, ketchup, and mustard should be restricted. Smoking and alcohol consumption should be prohibited. Sweet, pastries and carbonated drinks should not be consumed. It is necessary that food is not too hot or cold. Curd and other cheeses can be eaten freely"

".As a nutritional treatment, a diet with little pulp, non-stimulant, energy, and other nutrients (especially protein, vitamins A, C, E, and iron) is sufficient, the number of meals is high, and the proportion of nutrients is low (85)."

(Malan et al., 2017)

Gastro-oesophageal reflux: an overview of the pharmacotherapeutic treatment options

"It is recommended that patients refrain from indulging in foods that could trigger the onset of dyspeptic symptoms, such as fats, alcohol, peppermint and spearmint. These foods may decrease LESP or increase transient lower oesophageal sphincter relaxation whereas spicy foods, orange juice, tomato juice and coffee have a direct irritant effect on the oesophageal mucosa. Smaller meals should preferably be taken more frequently so as to avoid unnecessary gastric distension"

". • Adding protein-rich meals to the diet (augments LESP)"

(Rosa et al., 2018)

The role of diet in the prevention and treatment of Inflammatory Bowel Diseases

"Dietary recommendations include patient advice to self-monitor and avoid foods that may worsen symptoms, eating smaller meals at more frequent intervals, drinking adequate amounts of fluids, avoiding caffeine and alcohol, taking vitamin/ mineral supplementations, eliminating dairy if lactose intolerant, limiting excess fat, reducing carbohydrates and reducing high-fibre foods during flares."

(Kuzmin et al., 2025)

Efficacy of a Low-FODMAP Diet on the Severity of Gastrointestinal Symptoms and Quality of Life in the Treatment of

Gastrointestinal Disorders—A Systematic Review of Randomized Controlled Trials

> *"LFD: Involved a restricted intake of foods containing fermentable oligosaccharides, monosaccharides, disaccharides, and polyols. Patients received a pamphlet outlining restricted foods and suitable alternatives"*

> *". GDA: Incorporated dietary recommendations from the BDA (Mckenzie et al., 2016), including limiting caffeine, alcohol, spicy foods, high-fat foods, and carbonated beverages. Additional guidelines emphasized consuming small, frequent meals, eating slowly in a calm environment, and avoiding chewing gum and sweeteners containing polyols. (Iacovou et al., 2015). TDA: Patients were instructed to reduce the intake of fatty and spicy foods, coffee, and alcohol and to follow a regular meal pattern of three meals per day. They were encouraged to avoid overeating or undereating and to eat calmly, chewing food thoroughly (Böhn et al., 2015)."*

(Dimitrova-Yurukova et al., 2022)

Diagnosis and management of irritable bowel syndrome-like symptoms in ulcerative colitis.

> *"Regarding the diet, standard recommendations include adhering to a regular meal pattern, reducing intake of insoluble fibers, alcohol, caffeine, spicy foods, and fat. It is recommended that people should drink at least 1.5-2 litres of water per day in order to ensure proper hydration"*

> *". (Shanahan, 2004) Avoiding raw fruits and vegetables limits trauma to the inflamed colonic and may lessen symptoms in patients with UC. A milk-free diet also may help but need not be continued if no benefit is noted. [39] Patients should have light meals; eating should be slow and effective. Reducing the amount of sugar-consistent foods and sweeteners may relieve flatulence, bloating, and diarrhea. (Shanahan, 2004)"*

(Pasqui et al., 2016)

Dietary Management in IBS Patients

> *"• Have regular meals during the day and take time to eat with calm"*

> *".• Drink at least 1500 ml of liquid/day, preferring water or other non-caffeinated drinks"*

> *".• No more than three cups per day of tea and coffee"*

> *".• Reduce alcohol and fizzy drinks"*

> *".• Limit the intake of high-fiber food; reduce the intake of "resistant starch" (RS) because they resist to digestion in the small intestine and reaches the colon intact"*

> *".• No more than three portions of fresh fruit per day"*

> *".• In case of diarrhea, it is advisable to avoid sorbitol, an artificial sweetener found in sugarfree sweets, light drinks, and in some diabetic and diet products"*

> *".• In case of wind and bloating, it is advisable to eat oats and linseeds"*

> *".People with IBS should be discouraged from eating insoluble fiber. If an increase in dietary fiber is advised, it should be soluble fiber."*

(Zhang et al., 2024)

Influence of a diet meal plan on pepsinogen I and II, gastrin-17, and nutritional status in gastric ulcer patients

> *"Those with sudden onset and serious symptoms were urged to eat easily digestible and light foods, avoid stimulating foods, and limit the intake of milk and strong tea;"*

> *"Convalescing patients were advised to eat more alkaline foods and increase their intake of zinc and foods that can remove free radicals, such as soy products, fresh fruits, and sweet potatoes, and eat less or no sweets"*

Condition-specific dietary strategies

The best diet depends on the condition causing the stomach or gut symptoms. For IBS, a low-FODMAP diet has the strongest support; for IBD, diet is often adjusted to symptom phase; and for a few specific disorders, targeted sugar or allergen elimination may be used. (23 sources)

- **For irritable bowel syndrome (IBS), a low-FODMAP diet is the main targeted diet to consider.** It is the most consistently supported diet for improving overall IBS symptoms, and several guidelines recommend a **limited trial** rather than permanent strict avoidance. The usual approach is short-term restriction, then

gradual reintroduction to find which FODMAP groups actually trigger symptoms. This works best with dietitian support because the diet is restrictive and can lead to unnecessary food avoidance if done alone. (Dieterich et al., 2019) (Nanayakkara et al., 2016) (Moshiree et al., 2022) (Lacy et al., 2020) (Ruggiero et al., 2023) (Staudacher et al., 2021) (Kang, 2025) (Butt et al., 2025) (Haghbin et al., 2024)

- **In IBS, start by targeting common symptom triggers before wider restriction.** Advice often includes reducing **gas-producing foods**, and in some patients trying **lactose avoidance** if milk seems to trigger symptoms. Lower intake of **fat, insoluble fiber, caffeine, bloating foods, spicy foods, alcohol, carbonated drinks**, and sometimes limiting fruit to about **3 portions a day** may also help. (Hadjivasilis et al., 2019) (Dieterich et al., 2019) (Ruggiero et al., 2023)
- **Do not assume gluten is the problem in IBS unless celiac disease is present.** A gluten-free diet is sometimes used, but it is **not routinely recommended** without celiac disease, because many wheat foods also contain **fructans**, which are FODMAPs and may be the real trigger. (Moshiree et al., 2022) (Vasant et al., 2021)
- **Some IBS patients may benefit from individualized fiber or starch-sugar changes.** Reviews note that tailored advice such as **more soluble fiber** and adjusting the balance of carbohydrate, fat, and protein can help symptoms. A few newer studies also tested **starch- and sucrose-reduced diets**, with less confectionery, sweetened dairy, processed foods, bread, pasta, and rice, and more meats, fish, eggs, natural dairy, nuts, seeds, and selected lower-starch fruits and vegetables. These are more condition-specific options, not universal first-line diets for everyone with stomach complaints. (Mazzawi et al., 2017) (Nilholm et al., 2021) (Roth et al., 2022)
- **For inflammatory bowel disease (IBD), use symptom-based adjustments rather than one fixed diet.** Many patients can eat a fairly normal diet except for foods that clearly worsen symptoms. During **flares**, diarrhea, pain, or if there is a **stricture**, a **low-residue / low-fiber** plan with **well-cooked, soft, bland foods** can be helpful; some patients also avoid milk if lactose intolerance is present. (Misra, 2014) (Bueno-Hernandez et al., 2025)
- **Some IBD programs use phased diets such as IBD-AID.** These plans aim to increase **probiotic foods** and **soluble-fiber prebiotic foods**, include lean proteins and healthier fats, and avoid foods some patients react to, such as **lactose, wheat, refined sugars, corn, trans fats, processed foods, and fast food**. In active flares, the texture is often changed to **soft-cooked, pureed, or blended foods**, and intact fiber, stems, and seeds may need to be limited, especially if strictures are present. (Ye et al., 2024) (Peter et al., 2020) (Olendzki et al., 2014)
- **For eosinophilic esophagitis (EoE), elimination diets are disease-specific and should be supervised.** Older six-food elimination plans removed major allergens such as **milk, egg, wheat, soy, peanut/tree nuts, fish, and shellfish**, but newer step-up approaches often start with **milk alone** or **milk plus wheat/gluten** to reduce treatment burden. (Thomas et al., 2025) (Wechsler et al., 2021)
- **For short bowel or malabsorption states, diet needs a different structure.** Suggested measures can include **avoiding hyperosmolar drinks**, controlling **simple sugars**, using **some starch-containing foods**, limiting **lactose**, choosing the **type of fiber based on symptoms**, lowering **fat**, and avoiding **oxalate** in some cases. This is a specialist area and usually needs individualized medical nutrition advice. (Solar et al., 2024)
- **If a formal exclusion diet is used, it should be temporary and structured.** Older IBS studies found benefit from supervised elimination diets, but the key step was always to **reintroduce foods one at a time** and identify the true trigger foods, instead of staying on a very restricted diet long term. (Thompson et al., 2013) (Hadjivasilis et al., 2019)

Evidence

(Dieterich et al., 2019)

Gluten and FODMAPS—Sense of a Restriction/When Is Restriction Necessary?

> *"the introduction of a low FODMAP (fermentable oligo-, di-, monosaccharides, and polyols) diet reduced gastrointestinal symptoms in patients with IBS and this diet is suggested as the first choice of therapy in IBS"*
>
> *".The underlying idea is to avoid foods with a high FODMAP content, resulting in reduced osmotic load and bacterial gas production to minimize gastrointestinal problems in hypersensitive individuals (Nanayakkara et al., 2016)"*
>
> *".usual dietary advise, which recommend regular meals and minor intake of fat, insoluble fibers, caffeine or bloating foods, and that both dietary approaches significantly reduced the severity of IBS symptoms (Böhn et al., 2015)."*

(Nanayakkara et al., 2016)

Efficacy of the low FODMAP diet for treating irritable bowel syndrome: the evidence to date

> *"This review summarizes the published clinical studies concerning the management of irritable bowel syndrome (IBS) using restriction of Fermentable Oligosaccharide, Disaccharide, Monosaccharide, and Polyols in the diet (low FODMAP diet). In recent years, the data supporting low FODMAP diet for the management of IBS symptoms have emerged, including several randomized controlled trials, case-control studies, and other observational studies. Unlike most dietary manipulations tried in the past to alleviate gastrointestinal symptoms of IBS, all studies on low FODMAP diet have consistently shown symptomatic benefits in the majority of patients with IBS. However, dietary adherence by the patients and clear dietary intervention led by specialized dietitians appear to be vital for the success of the diet. Up to 86% of patients with IBS find improvement in overall gastrointestinal symptoms as well as individual symptoms such as abdominal pain, bloating, constipation, diarrhea, abdominal distention, and flatulence following the diet. FODMAP restriction reduces the osmotic load and gas production in the distal small bowel and the proximal colon, providing symptomatic relief in patients with IBS. Long-term health effects of a low FODMAP diet are not known; however, stringent FODMAP restriction is not recommended owing to risks of inadequate nutrient intake and potential adverse effects from altered gut microbiota. In conclusion, the evidence to date strongly supports the efficacy of a low FODMAP diet in the treatment of IBS. Further studies are required to understand any potential adverse effects of long-term restriction of FODMAPs."*

(Moshiree et al., 2022)

A Narrative Review of Irritable Bowel Syndrome with Diarrhea: A Primer for Primary Care Providers

> *"The ACG guideline for IBS management recommends a limited trial of a low FODMAP diet to evaluate improvement in global IBS symptoms, and this is the only diet endorsed by the ACG guideline committee (Table 2) (Lacy et al., 2020)(Vasant et al., 2021)[56]"*
>
> *".Although gluten-containing foods have been associated with diarrhea, abdominal cramping, and bloating, a gluten-free diet is not routinely recommended in the absence of celiac disease, and food containing glutens (e.g., wheat-based foodstuffs) often contain fructans, a FODMAP constituent (Vasant et al., 2021)(Skodje et al., 2017)."*

(Lacy et al., 2020)

ACG Clinical Guideline: Management of Irritable Bowel Syndrome.

> *"Irritable bowel syndrome (IBS) is a highly prevalent, chronic disorder that significantly reduces patients' quality of life. Advances in diagnostic testing and in therapeutic options for patients with IBS led to the development of this first-ever American College of Gastroenterology clinical guideline for the management of IBS using Grading of Recommendations, Assessment, Development, and Evaluation (GRADE) methodology. Twenty-five clinically important questions were assessed after a comprehensive literature search; 9 questions focused on diagnostic testing; 16 questions focused on therapeutic options. Consensus was obtained using a modified Delphi approach, and based on GRADE methodology, we endorse the following: We suggest that a positive diagnostic strategy as compared to a diagnostic strategy of exclusion be used to improve time to initiating appropriate therapy. We suggest that serologic testing be performed to rule out celiac disease in patients with IBS and diarrhea symptoms. We suggest that fecal calprotectin be checked in patients with suspected IBS and diarrhea symptoms to rule out inflammatory bowel disease. We recommend a limited trial of a low fermentable oligosaccharides, disacchardies, monosaccharides, polyols (FODMAP) diet in patients with IBS to improve global symptoms. We recommend the use of chloride channel activators and guanylate cyclase activators to treat global IBS with constipation symptoms. We recommend the use of rifaximin to treat global IBS with diarrhea symptoms. We suggest that gut-directed psychotherapy be used to treat global IBS symptoms. Additional statements and information regarding diagnostic strategies, specific drugs, doses, and duration of therapy can be found in the guideline."*

(Ruggiero et al., 2023)

The Role of the Nurse in the Management of Irritable Bowel Syndrome: A Narrative Review

> *"One of the most recommended diets is the low FODMAP diet. FODMAP stands for fermentable oligosaccharides, disaccharides, monosaccharides, and polyols, which are short-chain carbohydrates (sugars) that the small intestine absorbs poorly, leading to impaired water reabsorption and gas production for fermentation by the gut microbiota in the colon. The low FODMAP diet involves a three-step process that should be guided by a dietitian, in which initially all FODMAP content is eliminated for four to six weeks*

and then individual groups of FODMAPs are gradually reintroduced to identify dose-dependent symptom triggers, ending with a phase in which the diet is individualized (Staudacher et al., 2021)"

".Foods such as dairy products, those containing incompletely absorbed carbohydrates, spicy and fatty foods, foods containing wheat, and those associated with histamine release have been shown to be associated with worsening GI symptoms in patients with IBS (Böhn et al., 2013)"

".reducing intake of foods that could cause symptoms (e.g., spicy, fatty, and processed foods; alcohol, caffeine, and carbonated beverages; commonly consumed gas-and fiber-producing foods; limit intake of fresh fruit to a maximum of three per day) (Mckenzie et al., 2016)."

(Staudacher et al., 2021)

Long-term personalized low FODMAP diet improves symptoms and maintains luminal Bifidobacteria abundance in irritable bowel syndrome

"Short-term trials demonstrate the low FODMAP diet improves symptoms of irritable bowel syndrome (IBS) but impacts nutrient intake and the gastrointestinal microbiota. The aim of this study was to investigate clinical symptoms, nutrient intake, and microbiota of patients with IBS 12 months after starting a low FODMAP diet."

(Kang, 2025)

[illegible]

"Many guidelines recommend a low-FODMAP diet for irritable bowel syndrome."

(Butt et al., 2025)

Irritable Bowel Syndrome in Inflammatory Bowel Disease: An Evidence-Based Practical Review

"Among dietary approaches for IBS, the low-FODMAP diet has the most robust evidence base (Haghbin et al., 2024). The low-FODMAP diet is a restrictive diet that first excludes all food"

"products containing FODMAPs and later re-introduces the FODMAPs one by one."

(Haghbin et al., 2024)

Efficacy of Dietary Interventions for Irritable Bowel Syndrome: A Systematic Review and Network Meta-Analysis

"Introduction: Irritable bowel syndrome (IBS) is a common condition that alters the quality of life of patients. A variety of dietary interventions have been introduced to address this debilitating condition. The low-FODMAP diet (LFD), gluten-free diet (GFD), and Mediterranean diet are examples showing efficacy. The aim of this network meta-analysis was to compare these interventions to find the best approach. Methods: We performed a systematic review of the available literature through 14 March 2024 in the following databases: Embase, PubMed, MEDLINE OVID, Web of Science, CINAHL Plus, and Cochrane Central. We only included randomized controlled trials (RCTs). We used a random effects model and conducted a direct meta-analysis based on the DerSimonian–Laird approach and a network meta-analysis based on the frequentist approach. Mean differences (MDs) with 95% confidence interval (CI) were calculated. The primary outcomes included IBS quality of life (IBS QOL) and IBS symptom severity scale (IBS-SSS). Results: We finalized 23 studies including 1689 IBS patients. In the direct meta-analysis, there was no statistically significant difference in any IBS score between GFD and either LFD or standard diet. Meanwhile, the LFD was statistically superior to the standard diet in the IBS-SSS (MD: −46.29, CI: −63.72−−28.86, p < 0.01) and IBS QOL (MD: 4.06, CI: 0.72–7.41, p = 0.02). On ranking, the Mediterranean diet showed the greatest improvement in IBS-SSS, IBS-QOL, distension, dissatisfaction, and general life interference, followed by the LFD alone or in combination with the GFD. Conclusions: We demonstrated the efficacy of dietary interventions such as the LFD and Mediterranean diet in improving IBS. There is a need for large RCTs with head-to-head comparisons, particularly for the Mediterranean diet."

(Hadjivasilis et al., 2019)

New insights into irritable bowel syndrome: from pathophysiology to treatment

"An acceptable initial therapy, especially for patients with mild disease, is lifestyle modification and education. This starts with removing gas-producing foods and by following a low-FODMAP diet (Table 6). If this is helpful, then reintroduction of foods, one at a time, is encouraged. Otherwise, these specific foods should be avoided in the long term (Zhu et al., 2013)"

".Lactose avoidance is also an option in some patients, both those with and without lactose intolerance, since it is possible that an ingredient in milk other than lactose triggers symptoms (Mckenzie et al., 2016)."

(Vasant et al., 2021)

British Society of Gastroenterology guidelines on the management of irritable bowel syndrome

"Irritable bowel syndrome (IBS) remains one of the most common gastrointestinal disorders seen by clinicians in both primary and

secondary care. Since publication of the last British Society of Gastroenterology (BSG) guideline in 2007, substantial advances have been made in understanding its complex pathophysiology, resulting in its re-classification as a disorder of gut-brain interaction, rather than a functional gastrointestinal disorder. Moreover, there has been a considerable amount of new evidence published concerning the diagnosis, investigation and management of IBS. The primary aim of this guideline, commissioned by the BSG, is to review and summarise the current evidence to inform and guide clinical practice, by providing a practical framework for evidence-based management of patients. One of the strengths of this guideline is that the recommendations for treatment are based on evidence derived from a comprehensive search of the medical literature, which was used to inform an update of a series of trial-based and network meta-analyses assessing the efficacy of dietary, pharmacological and psychological therapies in treating IBS. Specific recommendations have been made according to the Grading of Recommendations Assessment, Development and Evaluation system, summarising both the strength of the recommendations and the overall quality of evidence. Finally, this guideline identifies novel treatments that are in development, as well as highlighting areas of unmet need for future research."

(Mazzawi et al., 2017)

Effect of diet and individual dietary guidance on gastrointestinal endocrine cells in patients with irritable bowel syndrome (Review)

> *"Providing individual dietary guidance about a low FODMAP intake, high soluble-fiber intake, and changing the proportions of protein, fat and carbohydrates helps to reduce the symptoms experienced by patients with IBS and to improve their quality of life"*

> *".several studies have shown that a low-FODMAP diet can improve the symptoms experienced by patients with IBS (Roest et al., 2013)(Ong et al., 2010)(Shepherd et al., 2006)(Gearry et al., 2009)(Halmos et al., 2014)(Shepherd et al., 2008)(Staudacher et al., 2012)(Staudacher et al., 2011)(Wilder-Smith et al., 2013)"*

> *"Many patients with IBS make a conscious choice to avoid certain foodstuffs, some of which belong to the FODMAPs group. However, they also tend to 'unknowingly' consume other foodstuffs that are rich in FODMAPs and avoid food sources that are important to their health (Ostgaard et al., 2012)."*

(Nilholm et al., 2021)

Assessment of a 4-Week Starch- and Sucrose-Reduced Diet and Its Effects on Gastrointestinal Symptoms and Inflammatory Parameters among Patients with Irritable Bowel Syndrome

> *"Patients received verbal as well as written dietary advice focused on starch-and sucrose reduction, and increased intake of certain fruits and vegetables, meat, fish, and dairy products. The dietary advice was modified from dietary guidelines for patients with congenital sucrase-isomaltase deficiency (CSID) [19]. Briefly, all sucrose-containing foods were to be avoided. Processed rice and pasta were discouraged and fiber-rich alternatives (≤1 serving daily) were allowed. Fiber-rich bread was recommended instead of white bread. Whole grains from oats, barley and bran were recommended instead of processed breakfast cereals. Pork, beef, lamb, fish, turkey, chicken, and egg could be ingested without any restrictions. Processed meat such as bacon, sausage and pies should be avoided if they were treated with sugar or starch. Dairy products could be ingested without restrictions, but natural products without added sugar were recommended. Butter and oil intake was unrestricted, but margarine should be avoided. Drinks in the form of milk, sugar-free soda and home-made juices were allowed, if the juice was made from recommended fruits. Regarding spices, salt, pepper and fresh herbs could be used unrestrictedly. The participants were encouraged to read the table of contents carefully for all products. Nuts and seeds were recommended in place of sugary snacks. Increased fat and/or protein intake and prolonged chewing was encouraged, to enhance salivary amylase breakdown of starch and delay gastrointestinal transport"*

> *".Patients were provided with visual aids to familiarize themselves with allowed and prohibited foods, including lists of suitable fruits and vegetables with less starch and sucrose content (Supplementary Materials, Tables S1 and S2)."*

(Roth et al., 2022)

A Starch- and Sucrose-Reduced Diet in Irritable Bowel Syndrome Leads to Lower Circulating Levels of PAI-1 and Visfatin: A Randomized Controlled Study

> *"The dietary advice given to the randomized intervention group (n = 80) primarily focused on starch and sucrose reduction, with decreased intake of foods such as confectionaries, sweetened dairy products, processed/ultraprocessed foods, bread, pasta, and rice. Instead, participants were advised to continue with or increase their intake of all meats and fish, fat, natural dairy products, eggs, nuts and seeds, and selected berries, fruits, and vegetables low in starch and sucrose (Table 1) [32]. Gluten and lactose contents in the ingested products were unrestricted. Fiber-rich bread, raw rice, and fiber-rich pasta were preferred instead of white bread and ordinary rice and pasta to delay the nutrient transport through the gastrointestinal tract. Adding fat and/or protein to starch-rich foods was also recommended to enhance starch tolerance by delayed gastrointestinal transport, and thus, longer exposure time to digestive intestinal enzyme activity in the small intestine. In general, participants were recommended to restrict their intake of fiber-rich cereals to a maximum of one serving per day. The patients were encouraged to eat slowly and chew their food properly to increase the secretion of amylase, which can contribute to degradation of starch."*

(Misra, 2014)

Integrative Therapies and Pediatric Inflammatory Bowel Disease: The Current Evidence

> *"There is no evidence that inflammation of the intestines is directly related to particular foods. However, certain foods may worsen*

symptoms due to food allergy or food intolerance. Many IBD patients follow a normal diet, but avoid those foods that tend to exacerbate their symptoms"

".Some patients with IBD avoid milk products, because they suffer from lactose intolerance or experience exacerbation of symptoms from milk products"

".A common conventional diet option for IBD patients is the low-fiber with low-residue diet. This includes decreasing the intake of raw foods, like fruits, vegetables and nuts, that add bulk residue to stool. This diet may be helpful during flares and for patients with bowel strictures."

(Bueno-Hernandez et al., 2025)

Nutrition in Inflammatory Bowel Disease: Strategies to Improve Prognosis and New Therapeutic Approaches

"During the active phase of Inflammatory Bowel Disease, especially in symptoms such as diarrhea, abdominal pain, or stricture, a low-residue diet is recommended, with well-cooked foods that are low in insoluble fiber and free of irritants. It is preferable to prioritize bland preparations such as rice, mashed potatoes, cooked carrots, boiled chicken, and stewed fruit"

".Avoiding foods that may exacerbate or complicate symptoms is recommended, and reintroducing foods should be gradual."

(Ye et al., 2024)

Effects of an anti-inflammatory diet (AID) on maternal and neonatal health outcomes in pregnant Chinese patients with inflammatory bowel disease treated with infliximab (IFX)

"The IBD-AID (Table 1) emphasizes the consumption of probiotic and prebiotic foods while avoiding certain carbohydrates that trigger inflammation (Peter et al., 2020). Probiotic foods include plain yogurt, fermented vegetables, and kimchi. Prebiotic foods are rich in soluble fiber and include oats, bananas, ground flaxseeds, chia seeds, garlic, onions, and asparagus. Good nutrition in this diet involves consuming a variety of fruits and vegetables in forms that the gut can tolerate, incorporating lean proteins and healthy fats, limiting saturated fats to less than 5 grams per serving (such as in meat and dairy products), and increasing the intake of quality fats from sources like nuts and olive oil. The diet avoids foods containing lactose, wheat, refined sugars, and corn, as well as trans fats, processed foods, and fast food"

".Phase 1 is designed for periods of active symptoms, such as disease flares or bleeding, and focuses on soft-cooked or pureed foods, including smoothies, cooked oats, pureed soups, pureed vegetables, plain yogurt, and ground lean meats."

(Peter et al., 2020)

A dietary intervention to improve the microbiome composition of pregnant women with Crohn's disease and their offspring: The MELODY (Modulating Early Life Microbiome through Dietary Intervention in Pregnancy) trial design

"Crohn's disease (CD), a type of inflammatory bowel disease (IBD), is a chronic condition of the gastrointestinal tract that is caused by the loss of mucosal tolerance towards the commensal bacteria resulting in inflammatory responses. It has long been postulated that the gut microbiota, a complex and dynamic population of microorganisms, plays a key role in the pathogenesis of IBD. Maternal diagnosis of IBD has been identified as the greatest risk factor for IBD in offspring increasing the odds of developing the disease >4.5-fold. Moreover, babies born to mothers with IBD have demonstrated reduced gut bacterial diversity. There is accumulating evidence that the early life microbiota colonization is informed by maternal diet within the 3rd trimester of pregnancy. While babies born to mothers with IBD would pose an ideal cohort for intervention, no primary prevention measures are currently available. Therefore, we designed the MELODY (Modulating Early Life Microbiome through Dietary Intervention in Pregnancy) trial to test whether the IBD-AID™ dietary intervention during the last trimester of pregnancy can beneficially shift the microbiome of CD patients and their babies, thereby promoting a strong, effective immune system during a critical time of the immune system development. We will also test if favorable changes in the microbiome can lead to a reduced risk of postpartum CD relapse and lower mucosal inflammation in the offspring. This study will help create new opportunities to foster a healthy microbiome in the offspring at high risk of other immune-mediated diseases, potentially reducing their risk later in life."

(Olendzki et al., 2014)

An anti-inflammatory diet as treatment for inflammatory bowel disease: a case series report

"The IBD-AID consists of lean meats, poultry, fish, omega-3 eggs, particular sources of carbohydrate, select fruits and vegetables, nut and legume flours, limited aged cheeses (made with active cultures and enzymes), fresh cultured yogurt, kefir, miso and other cultured products (rich with certain probiotics) and honey. Prebiotics, in the form of soluble fiber (containing betaglucans and inulin, such as bananas, oats, blended chicory root, and flax meal) are suggested. In addition, the patient is advised to begin at a texture phase of the diet matching with symptomology, starting with phase one if in an active flare. Many patients require foods to be softened and textures mechanically altered by pureeing the foods, and avoiding foods with stems and seeds when starting the diet"

"intact fiber can be problematic for those with strictures and highly active mucosal inflammation. Some patients will require lifelong avoidance of intact fiber. Food irritants are not limited to intact fiber, but may include certain foods, processing agents and flavorings to which IBD patients may be reactive."

(Thomas et al., 2025)

A multidisciplinary approach to eosinophilic esophagitis management

> *"The six-food elimination diet (elimination of the top eight allergen groups: milk, egg, wheat, soy, peanut, tree nuts, fish, and shellfish) emerged in the 2000s as a popular treatment option. (Kagalwalla et al., 2006) More recently, step-up approaches beginning with less-restricted two-food (milk and wheat/gluten) or single-food (milk) elimination diets have been increasingly implemented in attempts to balance efficacy with the burden of treatment while limiting the need for repetitive esophagogastroduodenoscopies and biopsies. (Molina-Infante et al., 2017)(Wechsler et al., 2021)"*

(Wechsler et al., 2021)

A Single-Food Milk Elimination Diet Is Effective for Treatment of Eosinophilic Esophagitis in Children

> *"BACKGROUND & AIMS: Cow's milk protein (CMP) is the most common trigger of inflammation in children and adults with eosinophilic esophagitis (EoE). We sought to assess the clinical, endoscopic, and histologic efficacy of dietary elimination of all CMP-containing foods in EoE. METHODS: We performed a prospective observational study in children with EoE treated with the 1-food elimination diet (1FED), excluding all CMP. Children and their caretakers were educated by a registered dietitian regarding dietary elimination of all CMP-containing foods, with substitutions to meet nutritional needs for optimal growth and development, and daily meal planning. Upper endoscopy with biopsies was performed after 8 to 12 weeks of treatment. The primary end point was histologic remission, defined as fewer than 15 eosinophils per high-power field. Secondary end points were symptomatic, endoscopic, and quality-of-life (QOL) improvements. RESULTS: Forty-one children (76% male; ages, 9 ± 4 years; 88% white) underwent 1FED education and post-treatment endoscopy with biopsies. Histologic remission occurred in 21 (51%) children, with a decrease in peak eosinophils per high-power field from a median of 50 (interquartile range, 35–70) to a median of 1 (interquartile range, 0–6; P < .0001). Endoscopic abnormalities improved in 24 (59%) patients, while symptoms improved in 25 (61%). Improved symptoms included chest pain, dysphagia, and pocketing/spitting out food. Parents perceived worse QOL, while children perceived improved QOL with the 1FED. CONCLUSIONS: One-food elimination of CMP-containing foods from the diet induced histologic remission in more than 50% of children with EoE and led to significant improvement in symptoms and endoscopic abnormalities. The ease of implementation and adherence supports the 1FED as first-line dietary treatment."*

(Solar et al., 2024)

Current Status of Chronic Intestinal Failure Management in Adults

> *"-Avoid hyperosmolar solutions, control simple sugars, avoid high concentrations of salt"*
>
> *".-Include starch-containing food"*
>
> *".-Control lactose (less than 20 g/day)"*
>
> *".-Control simple sugars"*
>
> *".-Avoid oxalate consumption"*
>
> *".-Select the type of fiber according to symptoms"*
>
> *".-Low-fat diet."*

(Thompson et al., 2013)

Comparison of the antibacterial activity of essential oils and extracts of medicinal and culinary herbs to investigate potential new treatments for irritable bowel syndrome

> *"Early work pioneered the use of an exclusion diet consisting of one meat, one fruit and distilled or spring water for a week. If the patient's IBS symptoms resolved, foods were reintroduced one at a time and any resulting IBS symptoms noted (Jones et al., 1982). Two thirds of the patients reported resolution of their symptoms on the exclusion diet, and wheat, corn, dairy products, coffee, tea and citrus fruits were found to provoke IBS symptoms, even in a double blind food challenge. A more relaxed exclusion diet consisting of fish and meat (apart from beef) and rice, and lacking dairy products, citrus fruits, yeast, tap water and caffeinated drinks, reduced hydrogen excretion by IBS patients in addition to resolving their symptoms (King et al., 1998). This exclusion diet has been described in more detail [48]. These and other exclusion diets have been found to be useful under medical supervision by NICE [37], which also recommended that it may be helpful for IBS patients to limit both their fibre intake, and to eat no more than three portions of fruit per day. A recent approach to dietary manipulation has rationalised knowledge of the foods that commonly provoke IBS symptoms with an understanding of their chemical composition, namely the adoption of a diet low in FODMAPs (fermentable oligosaccharides, disaccharides, monosaccharides and polyols) (Gibson et al., 2010). This involves avoiding 1) fruits that are high in fructose, 2) dairy products apart from butter and hard cheese, because they contain lactose, 3) vegetables, legumes and cereals (including wheat) that contain the oligosaccharides fructans or galactans and 4) fruits, vegetables and artificial sweeteners that contain polyols [49]."*

Recommended foods and foods to prefer

In many stomach or gut problems, the safest foods to prefer are simple, soft, lower-fat, and easy to digest. Commonly favored choices include lean proteins, rice or oats, cooked vegetables, bananas, yogurt or lactose-free dairy if tolerated, and fluids such as water or clear soups. (7 sources)

- **Choose lean, lower-fat protein foods.** Good options often include **lean meat, fish, poultry, eggs**, and in some cases **beans** if they are tolerated. For chronic gastritis, moderate amounts of **lean beef or lean lamb** may be used, while fattier cuts are less suitable. (Xu et al., 2023) (Malinowski et al., 2020)
- **Use simple starches and grains that are easy on the stomach.** Common examples include **rice, potatoes, noodles, oats, crackers, toast**, and in low-FODMAP plans, **gluten-free grains such as rice**. These are especially often used when diarrhea or active symptoms are present. (Graves, 2013) (Malinowski et al., 2020) (Bueno-Hernandez et al., 2025)
- **Prefer soft, bland, well-cooked foods during flares.** When symptoms are active, foods such as **mashed potatoes, cooked carrots, boiled chicken, stewed fruit, cooked oats, pureed soups, pureed vegetables, smoothies, and ground lean meats** are commonly recommended because they are easier to tolerate. (Bueno-Hernandez et al., 2025) (Ye et al., 2024) (Peter et al., 2020)
- **Choose fruits and vegetables in forms you tolerate well.** Papers commonly mention **bananas** as a well-tolerated fruit, and recommend using a **variety of fruits and vegetables** with the texture adjusted to symptoms, such as cooked, stewed, blended, or pureed forms when needed. (Malinowski et al., 2020) (Ye et al., 2024) (Peter et al., 2020) (Bueno-Hernandez et al., 2025)
- **If using a low-FODMAP approach, favor the foods allowed within that plan.** Examples given include **meat, poultry, fish, eggs, lactose-free dairy, rice, bananas, spinach, most nuts, and most herbs and spices**. (Malinowski et al., 2020)
- **If dairy is wanted, use the type that matches tolerance.** Some patients do better with **lactose-free dairy**, while others may tolerate **plain yogurt** better than milk. Fermented dairy is also used in some IBD-focused diets. (Malinowski et al., 2020) (Ye et al., 2024) (Peter et al., 2020)
- **Fermented and probiotic foods may help some patients, especially in IBD-style plans.** Examples mentioned include **plain yogurt, fermented vegetables, and kimchi**. (Ye et al., 2024) (Peter et al., 2020)
- **Include soluble-fiber and prebiotic foods if they are tolerated.** Examples listed in IBD-AID-type guidance include **oats, bananas, ground flaxseeds, chia seeds, garlic, onions, and asparagus**, though tolerance can vary and some of these are not suitable in a low-FODMAP phase. (Ye et al., 2024) (Peter et al., 2020)
- **Use healthy fats in modest amounts rather than heavy, greasy foods.** Some plans favor **nuts and olive oil** and suggest keeping **saturated fat** low. (Ye et al., 2024) (Peter et al., 2020)
- **During short episodes of diarrhea or stomach upset, start simply.** A short period of **clear liquids with electrolyte replacement**, then foods like **bananas, rice, applesauce, toast, soup, oats, potatoes, and crackers**, is a common practical approach. (Graves, 2013)
- **Fresh, minimally processed foods are generally preferred.** Some recent guidance suggests increasing **fresh fruits, vegetables, and fermented foods like yogurt**, while reducing reliance on heavily processed foods. (Singhal et al., 2025)

Evidence

(Xu et al., 2023)

MedGPTEval: A Dataset and Benchmark to Evaluate Responses of Large Language Models in Medicine

"For patients with chronic gastritis, it is recommended to control the intake of red meat, but not completely. Moderate consumption of lean beef or lean lamb is possible, but high-fat and greasy meats such as fatty beef and fatty lamb should be avoided. Coffee stimulates gastric acid secretion and is not very friendly to people with chronic gastritis, but it is not completely unacceptable. It is recommended to drink it in moderation, preferably with some milk or sugar to neutralize the stimulation of stomach acid"

". you are advised to pay attention to control your diet in your daily life and avoid excessive intake of foods high in fat, sugar, and salt, as well as too much caffeine. You can choose foods that are low in fat, high in fiber and high in protein, such as lean meat, fish, beans, vegetables, and fruits."

(Malinowski et al., 2020)

The Rundown of Dietary Supplements and Their Effects on Inflammatory Bowel Disease—A Review

"The following foods are recommended when following a low FODMAP diet: meat, poultry, fish, eggs, lactose-free dairy, gluten-free grains (i.e., rice), fruit (i.e., bananas), vegetables (i.e., spinach), most nuts, most spices and herbs.Avoiding foods that have high lactose content, legumes, garlic, and onion."

(Graves, 2013)

Acute Gastroenteritis

"A short period of clear liquids with adequate electrolyte replacement is generally ideal"

".In patients with watery diarrhea, boiled rice, potato, noodles or oats with salt, soup, crackers, or bananas are recommended. This is often referred to as the BRAT diet-bananas, rice, applesauce, and toast. Avoid high-fat foods until normal bowel function returns. Secondary lactose malabsorption or intolerance occurs after infectious gastroenteritis and may last for several weeks, so avoiding lactose-containing foods during this time is appropriate. (Dupont, 1997)"

(Bueno-Hernandez et al., 2025)

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(Ye et al., 2024)

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inflammation in the offspring. This study will help create new opportunities to foster a healthy microbiome in the offspring at high risk of other immune-mediated diseases, potentially reducing their risk later in life."

(Singhal et al., 2025)

[A narrative review on fecal microbiota transplantation routes in ulcerative colitis: identifying the optimal approach across key parameters]

"Key recommendations include reducing gluten-based grains, refined sugars, and processed meats while increasing fresh fruits, vegetables, and fermented foods like yogurt"

".Eliminating processed and red meat while boosting the intake of fruits, vegetables, and fermented foods can enhance the effectiveness of FMT. However, cruciferous vegetables high in aryl hydrocarbon receptor (AhR) ligands should be avoided because of their potential negative effects on gut health. Focusing on nutrient-rich foods that provide beta-carotene, calcium, and magnesium is recommended, as poorly absorbed carbohydrates and fibers may worsen symptoms in patients with FMT."

Foods and beverages commonly avoided or limited

Foods most often limited are those that clearly trigger symptoms, especially high-FODMAP foods, insoluble fiber, very fatty meals, alcohol, coffee, spicy foods, and fizzy drinks. Dairy is not always a problem, but milk or other dairy may need to be limited if lactose intolerance is present. (10 sources)

- **High-FODMAP foods are one of the most common groups to limit, especially in IBS.** These include foods rich in poorly absorbed sugars such as **wheat, onions, garlic, pulses/legumes, milk, honey, apples, pears, and some sweeteners like sorbitol**. These foods can worsen bloating, pain, gas, and diarrhea in some patients. (Mazzawi et al., 2015) (Kurin et al., 2020) (Agarwal, 2022) (El-Salhy et al., 2015) (Roth et al., 2025) (Mazzawi et al., 2014)
- **Foods high in insoluble fiber may need to be reduced if they worsen symptoms.** Several IBS-focused papers specifically mention **insoluble fiber** as a symptom trigger and advise shifting away from foods with higher amounts of it when symptoms are active. (Mazzawi et al., 2015) (Agarwal, 2022) (El-Salhy et al., 2015) (Mazzawi et al., 2014)
- **Gas-producing foods are often limited when bloating is a major problem.** Common examples listed across papers include **beans or legumes, onions, cauliflower, celery, broccoli, wheat bran, carrots, raisins, bananas, apricots, prunes or plums**. Not all of these cause trouble for every person, but they are common foods to test as possible triggers. (Pessarelli et al., 2022) (Vahedi et al., 2010)
- **Very fatty, greasy, and highly processed foods are often cut back.** Guidance commonly suggests limiting **high-fat foods**, and some newer diet strategies also avoid **processed meats treated with sugar or starch** such as **bacon, pies, and sausage**, as well as heavily processed breakfast cereals. (Vahedi et al., 2010) (Roth et al., 2025)
- **Alcohol, coffee, and carbonated drinks are commonly limited.** These are repeatedly listed as drinks that can worsen gut symptoms, especially bloating, pain, reflux-like discomfort, or diarrhea in sensitive patients. (Pessarelli et al., 2022) (Roncoroni et al., 2022)
- **Spicy foods are commonly avoided if they trigger symptoms.** They are among the foods most often reported as troublesome in functional bowel symptoms and are frequently restricted in practical diet advice. (Roncoroni et al., 2022) (Pessarelli et al., 2022)
- **Dairy may need to be limited, but only in the right patients.** If a person has **lactase deficiency or lactose intolerance**, dairy can worsen symptoms and may need to be avoided; however, this does **not** mean **all** patients with stomach problems should cut out dairy, since some can tolerate **lactose-free dairy** well. (Vahedi et al., 2010) (Mazzawi et al., 2015) (Roth et al., 2025)
- **Some condiments and pickled foods may also irritate symptoms in some people.** Older practical guidance for IBS specifically lists **vinegar, mustard, ketchup, and pickled foods** as items some patients may want to avoid. (Vahedi et al., 2010)

- **In starch- and sucrose-reduction approaches, sugary foods are limited.** These plans ask patients to avoid **sucrose-containing foods** and reduce starch-heavy refined foods, while using less processed grain choices instead. (Roth et al., 2025)
- **The most useful rule is to limit foods that repeatedly trigger your own symptoms, not to avoid every "bad" food forever.** Studies note that patients often avoid many foods, sometimes even during remission, so it is better to use a structured, personal approach rather than broad long-term restriction without a clear reason. (Roncoroni et al., 2022) (Pessarelli et al., 2022) (Algera et al., 2019)

Evidence

(Mazzawi et al., 2015)

Dietary guidance normalizes large intestinal endocrine cell densities in patients with irritable bowel syndrome

> *"the patients received general information about IBS, with emphasis on the importance of a regular and healthy eating pattern and on the food items that worsen IBS symptoms such as insoluble dietary fiber and poorly absorbable FODMAPs (fermentable oligosaccharides, disaccharides, monosaccharides and polyols). The patients were informed that lactose-free milk and other lactosefree dairy products do not provoke IBS symptoms and hence they were allowed to consume such products daily during the study"*

> *". the patients were asked to alter their diet proportions of protein, fat and carbohydrate, avoid FODMAPs-rich items as well as insoluble fiber and to consume vegetables and fruits that contained lower amounts of FODMAPs and insoluble fiber."*

(Kurin et al., 2020)

Irritable bowel syndrome with diarrhea: Treatment is a work in progress.

> *"FODMAPs. Elimination of fermentable oligosaccharides, disaccharides, monosaccharides and polyols (FODMAPs) is recommended by the guidelines. (Cashman et al., 2016) FODMAPs are sugars that ferment in the gut due to inadequate digestion; common ones are lactose, fructose, fructans, and sorbitol. Foods containing FOD-MAPs include wheat, some fruits and vegetables, corn syrup, and onions."*

(Agarwal, 2022)

Lifestyles and their Close Relationship with Gastrointestinal Diseases (Part I: Diet)

> *"IBS patients often find intolerance to several food items [78,(Ostgaard et al., 2012). Studies have shown that the intake of low-fermentable oligo-, di-, monosaccharides, and polyols (FODMAP) foods lead to a positive clinical response in 50%-80% of IBS patients. These patients find improvements in several symptoms including bloating, flatulence, diarrhea, and chronic abdominal pain (El-Salhy et al., 2015)(Böhn et al., 2015). FODMAP also improves the QoL in these patients (Schumann et al., 2018)(Dionne et al., 2018). The list of foods that are high or low in FODMAP is extensive and can be obtained by visiting the website of the American College of Gastroenterolgy [84]"*

> *".There is an increased risk of IBD, especially UC among people who consume greater amounts of n-6 polyunsaturated fatty acids (PUFA)and a lower risk among people with diets high in fiber, fruits"*

(El-Salhy et al., 2015)

Diet in irritable bowel syndrome

> *"Irritable bowel syndrome (IBS) is a common chronic gastrointestinal disorder that is characterized by intermittent abdominal pain/discomfort, altered bowel habits and abdominal bloating/distension. This review aimed at presenting the recent developments concerning the role of diet in the pathophysiology and management of IBS. There is no convincing evidence that IBS patients suffer from food allergy/intolerance, and there is no evidence that gluten causes the debated new diagnosis of non-coeliac gluten sensitivity (NCGS). The component in wheat that triggers symptoms in NCGS appears to be the carbohydrates. Patients with NCGS appear to be IBS patients who are self-diagnosed and self-treated with a gluten-free diet. IBS symptoms are triggered by the consumption of the poorly absorbed fermentable oligo-, di-, monosaccharides and polyols (FODMAPs) and insoluble fibre. On reaching the distal small intestine and colon, FODMAPS and insoluble fibre increase the osmotic pressure in the large-intestine lumen and provide a substrate for bacterial fermentation, with consequent gas production, abdominal distension and abdominal pain or discomfort. Poor FODMAPS and insoluble fibres diet reduces the symptom and improve the quality of life in IBS patients. Moreover, it changes favourably the intestinal microbiota and restores the abnormalities in the gastrointestinal endocrine cells. Five gastrointestinal endocrine cell types that produce hormones regulating appetite and food intake are abnormal in IBS patients. Based on these hormonal abnormalities, one would expect that IBS patients to have increased food intake and body weight gain. However, the link between obesity and IBS is not fully studied. Individual dietary guidance for intake of poor FODMAPs and insoluble fibres*

diet in combination with probiotics intake and regular exercise is to be recommended for IBS patients."

(Roth et al., 2025)

[Dietary Modifications in IBS Leads to Reduced Symptoms, Weight, and Lipid Levels: Two Randomized Clinical Trials]

> *"Patients randomized to the SSRD had to focus on the reduction of starch and sucrose, and increased intake of meat, fish, dairy products, and fruits and vegetables low in starch"*
>
> *"Briefly, sucrose-containing foods should be avoided. One serving per day was allowed for whole-grain bread or oatmeal porridge. Whole grains from oats, barley, and bran were recommended instead of processed breakfast cereals. Fiber-rich pasta and rice should be chosen. Small amounts of processed rice and pasta once daily could be consumed for participants not tolerating fibers. Pork, beef, lamb, fish, turkey, chicken, and egg could be ingested without any restrictions. Processed meat treated with sugar or starch such as bacon, pies, and sausage should be avoided. Natural dairy products could be ingested without restrictions, whereas oat milk and soy milk were not allowed"*
>
> *". Participants randomized to the low FODMAP received written and oral information about the diet. They were informed to avoid or reduce their intake of fructans (e.g., wheat, onion, garlic), galacto-oligosaccharides (e.g., pulses (pea, lentil, faba bean, and lupin)), lactose (e.g., milk), fructose more than glucose (e.g., honey), and polyols (e.g., apples, pears) during the 4-week intervention [24]."*

(Mazzawi et al., 2014)

[Increased gastric chromogranin A cell density following changes to diets of patients with irritable bowel syndrome]

> *"The patients were then advised to alter the proportions of protein, fat and carbohydrate, avoid items rich in FODMAPs and insoluble fibers, and consume vegetables and fruits containing less FODMAPs and insoluble fibers."*

(Pessarelli et al., 2022)

[The low-FODMAP diet and the gluten-free diet in the management of functional abdominal bloating and distension]

> *"A possible approach for FABD patients is the elimination of gas-producing foods (e.g., onions, legumes, cauliflower, celery, bananas, apricots, prunes, broccoli, and wheat bran) and the recommendation of healthy eating habits [e.g., regular meals; restriction of alcohol, coffee, and spicy foods (Hookway et al., 2015)(Mckenzie et al., 2016)]. In addition, patients are often advised to eliminate foods that trigger their GI symptoms (Algera et al., 2019)."*

(Vahedi et al., 2010)

[Irritable Bowel Syndrome: A Review Article]

> *"Patients should be educated on how best to consume their three daily meals, by partaking of non-processed and fresh foods that consist of whole grains, fibers and vitamins two or three times a day, 60"*
>
> *"People who have both IBS and lactase deficiency should avoid dairy products. People with bloating and increased gas (flatulence) should try to avoid foods such as beans, onions, celery, carrots, raisins, bananas, apricots and plums. It is recommended that foods containing vinegar, mustard, ketchup and pickled foodstuffs not be consumed either, 60-62 and iv) In essence, IBS patients should avoid foods that trigger an onset of their symptoms, con-sume a minimum of high fat foods and take part in regular physical activity. 61"*

(Roncoroni et al., 2022)

[Nutrition in Patients with Inflammatory Bowel Diseases: A Narrative Review]

> *"Experts' opinions support 'healthy diets', including the consumption of plant-based instead of animal-derived foods and whole-foods instead of refined-foods"*
>
> *".The most avoided foods were spicy foods, dairy products, alcohol, fruits and vegetables and carbonated beverages; these foods were also avoided during remission and to prevent relapse."*

(Algera et al., 2019)

[The Dietary Management of Patients with Irritable Bowel Syndrome: A Narrative Review of the Existing and Emerging Evidence]

> *"Even though irritable bowel syndrome (IBS) has been known for more than 150 years, it still remains one of the research challenges of the 21st century. According to the current diagnostic Rome IV criteria, IBS is characterized by abdominal pain associated with defecation and/or a change in bowel habit, in the absence of detectable organic causes. Symptoms interfere with the daily life of patients, reduce health-related quality of life and lower the work productivity. Despite the high prevalence of approximately 10%, its pathophysiology is only partly understood and seems multifactorial. However, many patients report symptoms to be meal-related and certain ingested foods may generate an exaggerated gastrointestinal response. Patients tend to avoid and even exclude certain food products to relieve their symptoms, which could affect nutritional quality. We performed a narrative paper review of the existing and emerging evidence regarding dietary management of IBS patients, with the aim to enhance our understanding of how to move*

towards an individualized dietary approach for IBS patients in the near future."

Personalization, reintroduction, and supervision

The best diet plan is the one that matches the cause of symptoms and the foods each person actually reacts to. If foods are removed, they should usually be added back in a planned way, and restrictive diets are safest with medical or dietitian support. (LLM Memory)

Diet for stomach or gut problems should be personal, not based on a long list of foods to avoid forever. A good next step after basic meal changes is to look for patterns, test likely triggers one at a time, and match the diet to the diagnosis. If a short-term elimination diet is used, the goal is usually not permanent restriction but a careful reintroduction phase to see which foods truly cause symptoms and which can stay in the diet. This helps reduce unnecessary food avoidance and lowers the risk of poor nutrition. Medical review is important if there are alarm features such as weight loss, bleeding, anemia, trouble swallowing, persistent vomiting, fever, dehydration, or symptoms that do not improve. A registered dietitian can help most when the diet is complex, symptoms are ongoing, or whole food groups are being removed, such as with low-FODMAP, allergy elimination, malabsorption, or IBD-related plans. (Model-Generated)